Solace Shadow Program

A 90 day shadow deployment at Baylor Scott & White Medical Center, McKinney. No cost. No patient contact. Nothing shown to any clinician. Prepared July 2026.

Solace is one system that covers the whole emergency department visit. Intake, triage support, documentation, hand off. It sits on top of Epic and replaces nothing you already run.

We are asking for one thing. Let us run Solace in shadow at McKinney for 90 days, then give you the results.

Shadow means Solace watches. It scores every patient in the department in real time. It displays nothing, alerts nobody, and writes nothing back into Epic. At the end you get a report. Nothing else changes in the meantime.

What you get at the end

An answer to a question most emergency departments cannot currently answer about themselves:

> **Of the patients at McKinney who ended up needing critical care, what acuity were they given at the door, and how long did it take before anyone recognised how sick they were?**

That number is your under triage rate and your recognition delay. It is a patient safety figure. Epic reporting does not produce it as standard. It belongs to you whether or not anything further happens between us.

Alongside it, for each of those patients, we show whether Solace would have raised its hand earlier, and by how long. If the answer is that we would not have helped, that is what the report will say.

Why this is worth 90 days of your attention

The largest study of triage accuracy done in the United States looked at 5,315,176 emergency department encounters across 21 hospitals.

32.2%	**3.3%**	**66%**
of encounters were mistriaged	were under triaged, given a lower acuity than warranted	sensitivity for spotting a critically ill patient at triage

A separate 2026 study of 173,168 encounters found 3.6% of patients deteriorated, and **45% of those deteriorations happened while the patient was still in the emergency department**. In the same body of work, only 40% of ordered vital signs were actually recorded.

> **We do not know McKinney's numbers.** Everything above is national published data. The figures on page 2 apply those rates to your published visit volume, which makes them an illustration of scale and not a claim about your department. Your real rate may well be better. McKinney is a Level III trauma centre and Baylor Scott & White publishes ED wait times far below national averages. Finding your actual number is the whole point of the program.

Scale at McKinney

McKinney sees **43,301 emergency department visits a year**, roughly 119 a day, across 192 beds, with Advanced Level III Trauma designation. Applying the national rates to that volume looks like this.

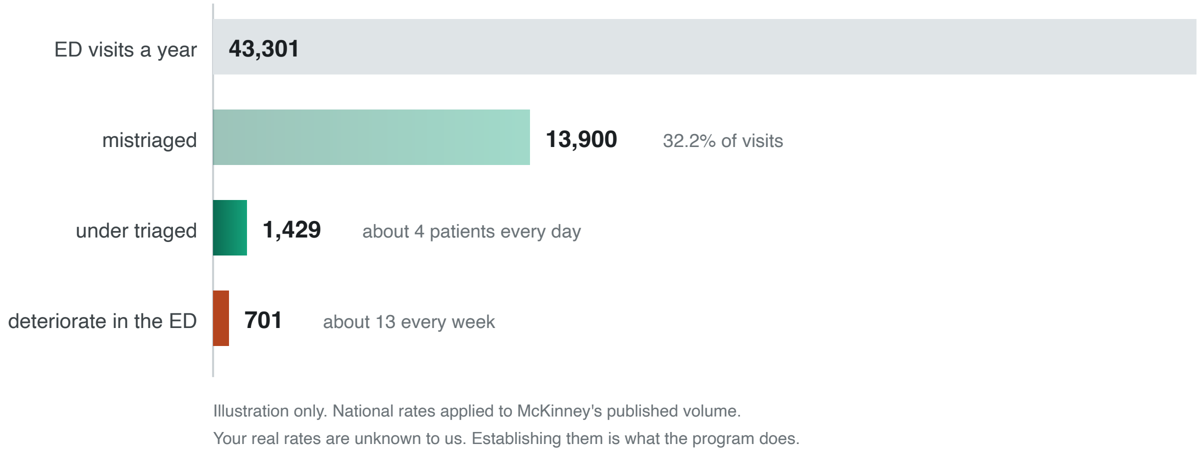


Figure 1. The size of the question at McKinney's volume.

Where the gap sits

Acuity gets assigned once, at the door, by a triage nurse, in a couple of minutes, on limited information. After that it is rarely revisited. A patient who gets under called at minute three carries that number for the rest of a stay that might run eight hours.

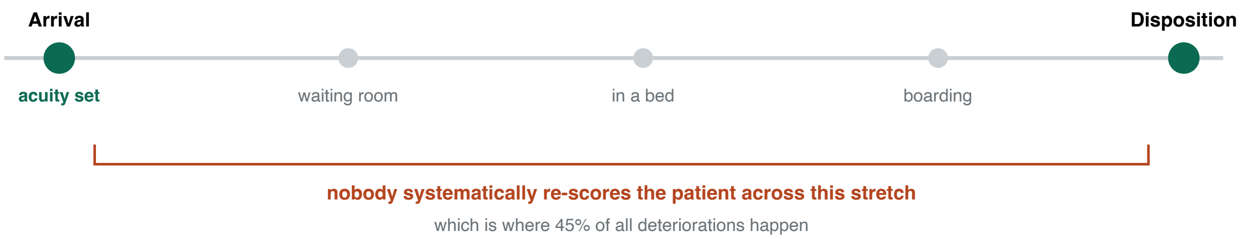
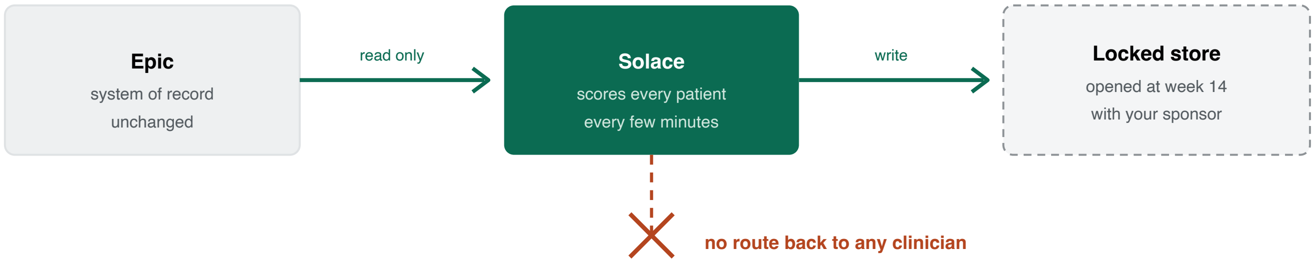


Figure 2. One assessment, then hours without another.

Closing that gap live is a clinical deployment and carries real risk, alert fatigue being the obvious one. We are not proposing that yet. We want to watch first, where the risk is zero, and let what we find decide whether anything more is justified.

How shadow mode works

This is the diagram your safety team will want. Data flows one way. Solace reads, scores, and writes to a locked store that only your sponsor and our team can open. No path exists from Solace back to a clinician, a screen, a pager, or Epic. There is no configuration in which it can interrupt anyone.



During the 90 days: no alerts, no dashboards, no orders, no notes, no messages, no changes to Epic.
Care at McKinney runs exactly as it does today. Solace is invisible to the department.

Figure 3. Shadow mode. The return path does not exist in the software, it is not switched off.

What the report looks like

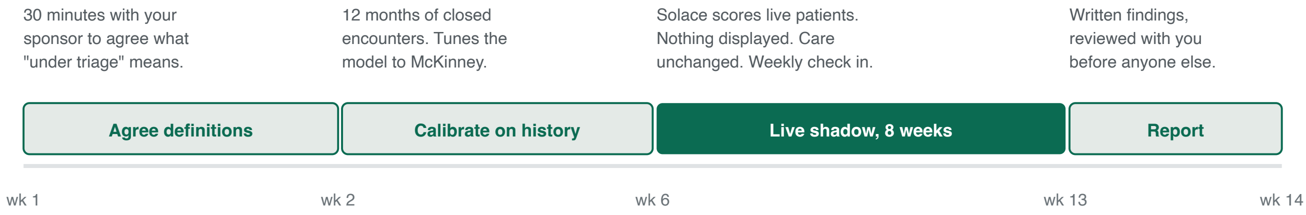
Row per patient, for everyone who ended up in critical care. Made up numbers below, real structure.

Case	ESI at door	Solace flagged	Team recognised	Lead time	Outcome
A-104	4	22 min	4 h 10 min	**3 h 48 min earlier**	ICU transfer
A-211	3	51 min	2 h 05 min	**1 h 14 min earlier**	ICU transfer
A-338	2	not flagged	18 min	no gain	already caught at triage
A-402	4	not flagged	6 h 30 min	**missed by us**	ICU transfer

The fourth row matters as much as the first three. Cases we miss go in the report.
So does every false alarm, counted against the number of patients seen.

Figure 4. Structure of the week 14 report. Misses and false alarms are reported, not buried.

The 90 days, week by week



Stop rule: you can end the program at any point, for any reason, without notice or explanation. Everything we hold is destroyed on request and we confirm it in writing.

Figure 5. Fourteen weeks from yes to answer.

What we need from Baylor Scott & White

What	Detail
A clinical sponsor	One emergency physician or nursing leader who will look at the findings and tell us honestly if they are useless. About two hours a month.
Read access to ED data	Whatever shape your privacy office is comfortable with. A limited data set under a DUA, a BAA, or the whole thing running inside your own environment so nothing leaves BSWH. We will build to whichever they pick.
Half an hour up front	To agree how under triage and deterioration get defined at McKinney, before we compute anything. Otherwise you get a number you do not recognise.
Not asked for	Money, an IT build, a workflow change, clinician training, or any commitment past week 14.

What we would ask for if the findings hold up

Permission to move to a supervised pilot, designed with your governance committee, with alert thresholds tuned from the shadow data and a stop rule agreed in advance. And if the work was genuinely useful, a reference we can show other Baylor Scott & White sites. Nothing beyond that at this stage.

> We expect a fair number of conversations like this to end at week 14, and that is a legitimate result. A department that learns its under triage rate is low has learned something worth knowing.

What Solace is, beyond this program

One system across the whole ED visit rather than a single feature bolted on. Epic stays the system of record. Solace reads and writes through standard interfaces, HL7 v2, FHIR R4 and CDS Hooks.

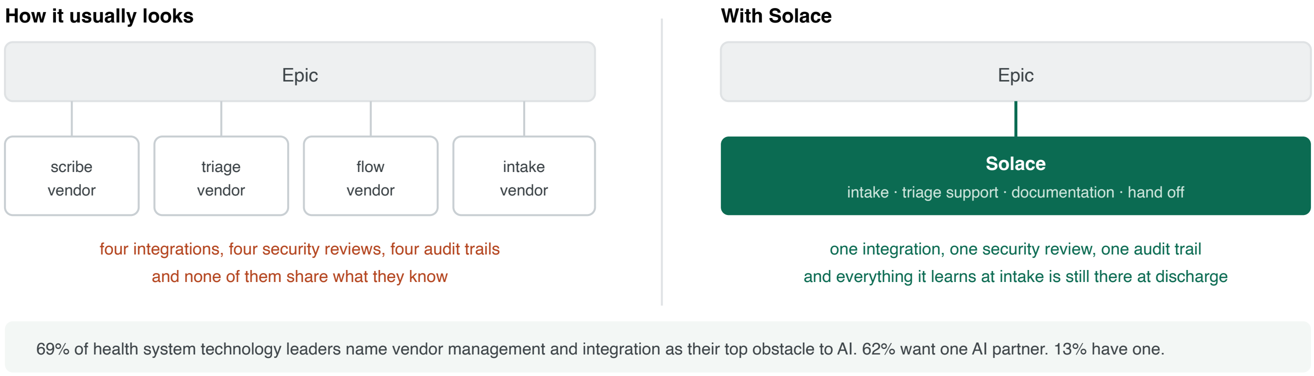


Figure 6. Solace does not replace Epic. It replaces the need for several contracts around it.

Two choices we made that we would like picked apart

- **The model says when it is unsure.** Instead of one acuity number it returns a calibrated range with a measured coverage rate. A wide range means get a person to look, not act on this.
- **Every output shows its working.** Each estimate comes with the specific inputs behind it. That is deliberate. FDA's January 2026 guidance on clinical decision support turns on whether a clinician can independently review the basis for a recommendation, and we built to that line rather than up against it.

Both choices cost accuracy on a leaderboard. In an emergency department we think a model that admits doubt beats one that is confidently wrong a third of the time.

Who we are

Solace is an early stage team. We have no hospital deployments behind us and we would rather tell you that than have you find it in a reference check.

What works today: multilingual patient intake, a calibrated triage model that reports its own uncertainty, ambient documentation, and interfaces to Epic, Oracle Health and athenahealth. There is a governance layer underneath it, model cards, consent enforcement and an append only audit log, because we built for a compliance review before we built for a demo.

> **What does not exist yet:** a completed clinical validation, a live deployment, or FDA clearance. Our triage model is trained on a public research dataset rather than real patient data. That gap is exactly why we are asking to run in shadow instead of asking you to switch something on. A model calibrated against McKinney's own encounters is worth more to both of us than anything we could claim today.

Why McKinney

- **Enough volume to mean something.** 43,301 visits a year gives enough events for the 90 days to produce a real answer rather than a suggestive one.
- **Level III trauma.** Acuity decisions carry weight, and there is already a quality structure that reviews them.
- **A good department is the harder test.** Baylor Scott & White publishes ED wait times four to six times shorter than national averages. If a signal shows up at McKinney it exists everywhere. If it does not, that is worth

knowing and we will say so.

> **The question we are asking:** may we run Solace in shadow at McKinney for 90 days, at no cost, with nothing shown to anyone, and then tell you your own under triage rate and how much earlier we would have caught the cases you missed?

References. Mistriage rates: Kaiser Permanente Division of Research, *JAMA Network Open*, n=5,315,176, pubmed.ncbi.nlm.nih.gov/36930151. ED deterioration: *Annals of Emergency Medicine*, 2026, n=173,168, annemergmed.com. CMS 2026 OPPS emergency care access and timeliness measure, ACEP. ONC HTI-1 decision support interventions, healthit.gov. FDA clinical decision support software guidance, January 2026, fda.gov. McKinney facility figures, bswhealth.com.